

pharmaniaga™

Salbutamol



Tablet



Syrup

DESCRIPTION

Pharmaniaga Salbutamol tablet 4 mg.

A pink, round, biconvex tablet with Pharmaniaga icon on one side and scoreline on the other.

Pharmaniaga Salbutamol syrup.

A clear, colourless syrup.

COMPOSITION

Each tablet contains Salbutamol (as sulphate) 4 mg.

Each 5 mL of syrup contains Salbutamol (as sulphate) 2 mg.

ACTIONS

A sympathomimetic agent with predominantly beta₂ - adrenergic activity. Being a selective beta₂ - adrenoceptor stimulant, it has a powerful bronchodilator action without much affecting the heart.

Relieves bronchospasm and increases vital capacity, sputum becomes thinner and is more mobile to be expectorated. Readily absorbed from the gastro-intestinal tract. About 50% of an oral dose is excreted in the urine within 4 hours

INDICATIONS

Salbutamol oral preparations are indicated for the relief of bronchospasm in bronchial asthma of all types, chronic bronchitis and emphysema.

CONTRAINDICATIONS

Although salbutamol tablets are used in the management of premature labour, salbutamol preparations should not be used for threatened abortion during the first or second trimesters.

Salbutamol oral preparations and beta-blocking drugs such as propranolol, should not usually be prescribed together.

Salbutamol preparations are contraindicated in patients with a history of hypersensitivity to any of their components.

PRECAUTIONS

As maternal pulmonary oedema and myocardial ischaemia have been reported during or following premature labour in patients receiving beta₂-agonists, careful attention should be given to fluid balance and cardio-respiratory function, including ECG monitoring. If signs of pulmonary oedema and myocardial ischaemia develop, discontinuation of treatment should be considered.

Due to the risk of pulmonary oedema and myocardial ischaemia that has been observed during the use of beta₂ - agonists in the treatment of premature labour, before these products are given to any patients with known heart disease, an

adequate assessment of the patients cardiovascular status should be made by a physician experienced in cardiology.

Salbutamol should be administered cautiously to patients suffering from thyrotoxicosis.

Tocolysis: Serious adverse reactions including death have been reported after administration of terbutaline/salbutamol to women in labour. In the mother, these include increased heart rate, transient hyperglycaemia, hypokalaemia, cardiac arrhythmias, pulmonary oedema and myocardial ischaemia. Increased fetal and neonatal hypoglycaemia may occur as a result of maternal administration.

Pregnancy and Lactation

Although salbutamol tablets are used in the management of premature labour, salbutamol preparations should not be used for threatened abortion during the first or second trimesters.

Unnecessary administration of drugs during the first trimester of pregnancy is undesirable.

ADVERSE REACTIONS

The only adverse reaction of significance with oral salbutamol is fine tremor of skeletal muscle which occurs in some patients, usually the hands are most obviously affected. This effect is dose related and is common to all beta-adrenergic stimulants. A few patients feel tense; this is also due to the effects on skeletal muscle and not to direct CNS stimulation. With doses of salbutamol higher than those recommended or in patients who are unusually sensitive to beta-adrenergic stimulants peripheral vasodilation and a compensatory small

increase in heart rate may occur. Occasionally headaches have been reported.

DOSAGE AND ADMINISTRATION

Tablet 4 mg and syrup 2 mg/5 mL.

Adults :

The usual effective dose is 2 - 4 mg of tablet or 5 - 10 mL of syrup three or four times per day. If adequate bronchodilation is not obtained each single dose may be gradually increased to as much as 8 mg.

However, it has been established that some patients obtain adequate relief with 2 mg three or four times daily. In elderly patients or in those known to be unusually sensitive to beta-adrenergic stimulant drugs, it is advisable to initiate treatment with 2 mg three or four times per day.

Children:

The following doses should be administered three or four times daily.

2 - 6 years : 1 - 2 mg as 2.5 - 5 mL of syrup

6 - 12 years : 2 mg as 5 mL of syrup

Over 12 years : 2 - 4 mg as 1 tablet of 4 mg

The drug is well tolerated by children so that, if necessary, these doses can be cautiously increased.

In the management of premature labour, after uterine contractions have been controlled by intravenous infusion of salbutamol and the infusion has been withdrawn, maintenance therapy can be continued with oral salbutamol. The usual dosage is 4 mg three or four times daily.

ROUTE OF ADMINISTRATION

Oral.

OVERDOSAGE

The preferred antidote for overdosage with salbutamol is a cardioselective beta-blocking agents, but beta-blocking drugs should be used with caution in patients with a history of bronchospasm.

STORAGE CONDITIONS

Pharmaniaga Salbutamol tablet 4 mg

Store below 30 °C.

Protect from light.

Pharmaniaga Salbutamol syrup

Store below 25 °C.

Keep container tightly closed.

Protect from light.

SHELF LIFE

Product should not be used beyond the expiry date imprinted on the product packaging.

PRESENTATION

Pharmaniaga Salbutamol tablet 4 mg.

In blisters of 10, 20, 30, 50, 100, 250, and 500 tablets.

Pharmaniaga Salbutamol syrup.

In bottles of 60 mL, 90 mL and 120 mL.

Date of revision: 14th February 2018

PRODUCT REGISTRATION HOLDER/

MANUFACTURER:

PHARMANIAGA MANUFACTURING BERHAD

(60016-D)

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